

Respiratory R-03 General Respiratory Distress BLS

Revised 3/1/2000

PRIORITIES:

- ABCs
- Determine degree of physiologic distress:
 - Respiratory rate >20, use of accessory muscles, cyanosis, inadequate ventilation, depressed level of consciousness
- Maintain airway, provide oxygen and ventilatory support;
- Determine which causes best fit patient signs and symptoms, initiate treatment;
- Assure an advanced life support response;
- Early transport after an EMS transporting unit arrives.

Respiratory Distress

Increased respiratory rate, sensation of difficult breathing. May be due to pneumonia, inhalation of toxic substances, pulmonary embolus.

1. Ensure a patent airway (suction as necessary);
2. Be prepared to support ventilation with appropriate airway adjuncts;
3. OXYGEN THERAPY – Begin oxygen at 6 liters/ minute by nasal cannula or 10 liters/minute by mask. If there is a history of Chronic Obstructive Pulmonary Disease (COPD), observe for respiratory depression and support respirations as needed. **DO NOT** withhold oxygen from a patient in cardiorespiratory distress because of a history of COPD.
4. Place patient in position of comfort if conscious. If depressed level of consciousness, position on left side;
5. Assist advanced life support personnel with patient packaging and movement to ambulance;

Respiratory

R-03 General Respiratory Distress

PRIORITIES:

- ABCs.
- Position of comfort PRN.
- Determine degree of physiologic distress.
- Evaluate respiratory rate, use of accessory muscles, cyanosis, ventilatory effort, level of consciousness.
- Maintain airway, provide oxygen and ventilatory support PRN.
- Consider 12 Lead EKG for patient age 35 or greater.
- Early transport (after initiating therapy), if appropriate.
- Consider possible causes including airway obstruction, pulmonary embolism, overdose, asthma, COPD, CHF, MI, pneumonia, pneumothorax, cardiac tamponade, anaphylaxis.
- **EARLY CONTACT OF RECEIVING HOSPITAL.**

Consider OXYGEN THERAPY – based on patient condition (consider lower dose in COPD patients).



Consider CARDIAC MONITOR.



Consider IV Access TKO.



Follow the specific protocol to treat specific causes of SOB.

Transport to appropriate facility.

Disrupted Communications

In the event of a "disrupted communication" situation, the EMT-P in Solano County may utilize all portions of this treatment protocol without Base Hospital contact as is needed to stabilize an immediate patient.